ElationHealth

2025-06-16

Centers for Medicare & Medicaid Services
Attention: CMS-0042-NC
Mail Stop C4-26-05
7500 Security Boulevard
Baltimore, MD 21244-1850

Re: Request for Information—Health Technology Ecosystem (RFI 2025-08701)

We appreciate the opportunity to respond to CMS and ASTP/ONC's Request for Information on the health technology ecosystem, digital health, and interoperability. Our comments are focused on specifically addressing macro-structural reform, payment, certification, and AI-enabled interoperability. As a leading EHR vendor and clinical workflow solution serving independent primary care organizations nationwide, including 36,000 clinical users caring for 20 million patients, Elation Health is uniquely positioned to provide insights and recommendations that marry frontline clinical experience with technical expertise and a deep understanding of policy and financial incentives.

Our recommendations to CMS for transforming the health technology ecosystem focus on aligning incentives to create market demand, reforming certification, streamlining quality reporting through administrative simplicity, modernizing interoperability to support innovation, and, most importantly, empowering primary care. The current regulatory approach fails to align with genuine clinical needs—especially the needs of primary care. The following comments synthesize our views:

Reform Market Incentives through Primary Care and Value-Based Models

The foundational problem of the current technology ecosystem is not technical but incentive misalignment. Technological progress alone will not solve data exchange and interoperability deficits. The real barrier is misaligned market incentives, especially payment structures that fail to reward longitudinal, coordinated, whole-person primary care and instead incentivize a system which benefits from an unhealthy population. Technology, no matter how sophisticated, won't yield meaningful information exchange unless there's a true market demand for interoperability. The most powerful lever CMS controls is payment, and payment reform, particularly for primary care, is the critical lever CMS can pull to promote the kind of robust interoperability that underpins overall health improvement at a lower total cost of care.

Recommendations:

- Directly increase prospective payments for primary care, focusing on value-based arrangements. Empowering primary care in this way creates organic, market-driven demand for comprehensive, longitudinal patient records and robust interoperability infrastructure.
- Recognize primary care as the central use case for interoperability—and as the greatest beneficiary. Structuring incentives for primary care will move the needle system-wide.

Certification Reform and Outcome Focus

Today's CEHRT requirements mandate many low-value features (e.g., clock synchronization), consuming vast vendor resources for elements that clinicians do not demand. The result is an ecosystem where real interoperability lags, and true clinical or customer needs are deprioritized. CMS should reframe certification to prioritize end-to-end utility for clinicians—delivering what matters most for care coordination and patient outcomes—rather than focusing on technical features without clear specific use cases.

Recommendations:

- Shift certification from prescriptive, technical checklists to clear, outcome-based mandates that focus on real-world, functional data interoperability.
- Discard requirements that would be "market-corrected" (e.g., basic system functions whose failure would render products unsellable).
- Consider certifying the real-world performance and reliability of APIs for end-to-end, high-value clinical use cases (e.g., successful referrals), not just theoretical availability of data.

This would align market incentives, empowering vendors and clinicians to strive for and be recognized for tangible advances in interoperability and care coordination, not just compliance and certification.

Streamline Quality Reporting to Reduce Administrative Burden

Today's approach to quality measurement is needlessly burdensome, requiring clinicians to contract with multiple vendors and utilize multiple disjointed systems just to satisfy variable payer definitions. CMS' bulk FHIR and related API requirements offer an underutilized opportunity: CMS should itself take on the task of quality measurement calculation, using the

existing data streams. Consolidation of reporting and interoperability responsibilities—leveraging bulk transfer and registry integration—can reduce complexity, if carefully implemented. CMS can and should build a pathway for FHIR bulk integration to measure quality across providers. Leveraging digital Quality Measures (dQMs) and consistently building emphasis on the FHIR pathway for quality reporting will allow for more consistency across payers to measure consistent data points from a standardized endpoint. This would ease the administrative burden on clinicians, set the benchmark for payers, and drive greater consistency and transparency across the industry. Today's fragmented, vendor-dependent reporting systems and misalignment across payers waste time and resources without improving care.

Recommendations:

- Centralize and standardize quality measure calculation at CMS using bulk FHIR data, relieving clinicians of downstream vendor dependence and redundant tool maintenance.
- Use this centralization as a template to drive industry-wide standardization, reduce waste, and let clinicians focus on care, not compliance.
- Standardize FHIR in a meaningful way by standardizing performance metrics.

Modernize Interoperability Standards: AI, FHIR, Proprietary APIs

AI is reshaping the conversation about standards fast, and the debate between FHIR and proprietary APIs is quickly losing relevance. While the FHIR standard remains an important "minimum viable" baseline and proprietary APIs support more complex use cases, advances in AI are flattening the differences. Soon, AI will serve as an instant translator across standards, creating structured actionable data out of "blobs of text". True interoperability can then focus less on rigid adherence to a single standard and more on simply ensuring the frictionless exchange of rich data - expanding the horizon for deeper interoperability and more innovative care tools. CMS should provide regulatory flexibility that supports innovation in interoperability. Standardized FHIR endpoints for regulatory reporting, such as quality measurement as described above, along with AI to translate clinical data to FHIR endpoints, remains an important pathway to reducing burdensome variation in documentation and reporting requirements at the point of care, reducing waste and freeing clinicians to promote health and tackle chronic disease.

Recommendations:

- Recognize the accelerating ability of AI to translate between standards and to structure unstructured records into usable, interoperable data.

- Prepare for an ecosystem where AI-mediated interoperability makes rigid adherence to a single data standard less essential.
- Encourage regulatory flexibility to allow diverse, innovative approaches, in addition to the current paths.

Reduce Red Tape, Clarify Liability, and Improve Digital Identity

Administrative friction—fear of liability, endless paperwork, and confusion around consent—slows genuine data sharing and is even being used to create proprietary moats around technology and delivery systems. CMS can make a tangible difference by explicitly clarifying, simplifying, and enforcing these rules, educating stakeholders, and fostering a sense of legal and practical confidence and accountability in data exchange.

Recommendations:

- Streamline, clarify, standardize, and enforce regulations on patient consent, identity verification, and liability.
- Invest in provider and stakeholder education to ensure rules are well understood and friction is minimized.

Conclusion

The greatest opportunities for U.S. health technology policy lie in aligning certification and payment incentives with genuine clinical needs—especially the needs of primary care. The current system-wide failures are not technology failures, but failures of economic alignment and systemic incentive fragmentation. The way CMS incentivizes care delivery is as important—perhaps more important—than how it regulates technologies. To get to a primary care–driven, health-promoting future, CMS must lead with incentives aligned for longitudinal, patient-centered care. Certification and technology policy should follow this north star, shedding distractions that do not serve clinical outcomes, and industry will respond to meet that demand.

We support CMS and ASTP/ONC's vision for a truly interoperable, patient-and-provider empowering digital health ecosystem. We urge CMS to:

- Align payment models to power longitudinal, whole-person primary care—especially in value-based arrangements—so market demand drives true interoperability.
- Make certification outcome-focused; eliminate low-value checklists and certify real-world, functional interoperability.
- Modernize interoperability policy recognizing the revolutionary impact of advances in AI

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- Centralize and standardize quality reporting at CMS to cut waste and free clinicians from reporting burdens.
- Demand real-world outcomes as the measure of policy success.

Thank you for your leadership and the opportunity to participate in this vital dialogue. We look forward to building a technology ecosystem that enables trusted, high-quality, and equitable care for all beneficiaries.

Sincerely,

Sara J. Pastoor, MD, MHA
Head of Primary Care Advancement, Elation Health

Cc:
Kyna Fong, PhD
CEO and Co-Founder
Elation Health